

Sleep regimen — old stack vs. new

A medium-term choice, honestly framed, with clear stacking rules · 2026-05-21

For Dad, Mom, Leah, David,
Debbi, family team

What this is for. The actual decision Dad and Mom are weighing on sleep — for the medium-term window (2–6 weeks) before ECT starts at U-M. The two real candidates are **lemborexant 5 mg + melatonin 10 mg** (the new agent Julane recommends) and **doxepin 6 mg + trazodone 50 mg + melatonin 10 mg** (the prior stack Dad was on, which produced 5.5–6 hours of "okay not great" sleep). After a focused evidence review, **the comparison is closer to a coin flip than to a clear winner** — the published evidence does not strongly favor either regimen, and the magnitude of the advantage for either side is small. Each has a modest evidence-supported edge on different axes. This handout lays out both honestly, plus the clear "do not combine" stacking rules so the choice is made deliberately rather than by drift.

BACKGROUND THAT HOLDS ACROSS EITHER CHOICE

- **Cymbalta 60 mg daily** — taken in the morning. Taper is paused; Dad is back at 60 mg as of 5/21 after the difficult overnight 5/20.
- **Depakote status TBD** — being discussed; not yet confirmed as part of the medium-term regimen. This handout treats Depakote as a separate decision and does not fold it into the sleep regimen question.
- **Melatonin 10 mg HS** — continues with either sleep regimen. Negligible risk; modest evidence; no interactions.
- **Lorazepam (Ativan) PRN** — available per Maxner clearance. Family preference is to avoid scheduled use given habit-formation concerns; a single PRN dose for an acute episode is acceptable.
- **ECT at U-M** is 2–4 weeks out. The medium-term window for the sleep regimen is approximately the next 2–4 weeks until ECT starts and then through the ECT course.

THE TWO REAL OPTIONS

Both are defensible. The honest evidence read is that this is closer to a coin flip than a clear winner — each option has a modest edge on different axes, and the magnitude of the advantage for either side is small. Read the pros and cons together and pick the one that better fits Dad's preferences, the medium-term trajectory, and the U-M Movement Disorders evaluation that's pending.

OPTION A — THE NEW STACK (JULANE'S RECOMMENDATION)

Lemborexant 5 mg HS + melatonin 10 mg HS

PROS

- **Real evidence base in elderly insomnia.** SUNRISE-1 (Rosenberg 2019, JAMA Network Open) showed improved sleep onset and sleep maintenance in adults ≥ 55 vs. placebo and zolpidem. SUNRISE-2 ≥ 65 subgroup (Arnold 2024) confirmed sustained benefit on subjective sleep, ISI scores, fatigue, and daytime functioning at months 1, 3, and 6.

OPTION B — THE OLD STACK

Doxepin 6 mg HS + trazodone 50 mg HS + melatonin 10 mg HS

PROS

- **Known-tolerated by Dad.** Personal data: 5.5–6 hours of sleep, no serious adverse effects, no overnight emergencies. The known floor is meaningful in a fragile moment.
- **Anticholinergic exposure at 6 mg $\times$ 2–6 weeks is quantitatively negligible.** By the Gray 2015 / Coupland 2019 cumulative-dose framework, doxepin 6 mg/day for 6 weeks accumulates ~ 25 standardized daily doses out of the ≥ 1095 threshold associated with elevated dementia risk — about 2% of the level where measurable harm has been demonstrated.

- **Rapid plateau, not progressive build.** The sleep effect is largely established within the first week and plateaus thereafter. **A 1–2 week trial is sufficient to assess whether it works for Dad** — we won't be waiting weeks for it to "kick in."
- **17-hour half-life provides built-in sleep maintenance.** The drug is still working at 2 AM — reduces wake-after-sleep-onset (WASO) in the second half of the night, which is precisely where Dad's prior stack underperformed (waking unrested).
- **No anticholinergic burden.** Lancet Neurology 2025's DLB therapeutic landscape review categorically recommends avoiding anticholinergic TCAs in Lewy-spectrum disease, without specifying a dose threshold. Lemborexant sidesteps that signal entirely.
- **Cleaner drug-interaction profile with Cymbalta.** No CYP2D6 interaction; no additive serotonergic load; no additive orthostasis. Lower fall-risk profile in Dad's bradycardic + low-BP + gait-abnormality setting.
- **No dependence risk, no withdrawal pattern.** Lowest abuse-potential profile of any insomnia agent per FAERS data — lower than trazodone and doxepin.
- **Single-agent simplicity.** One sleep agent + melatonin is easier to track, attribute, and adjust than two concurrent sleep agents + melatonin.

CONS

- **Untried by Dad.** No personal data on whether it works for him. Trial efficacy in elderly cohorts doesn't guarantee his individual response.
- **REM enhancement could unmask currently-quiescent RBD episodes.** Lemborexant significantly increases REM duration and decreases REM latency. Dad's RBD episodes have been declining over the past year (most recent ~Feb 2026, now every few months, no dream recall recently) — but the current quiescence could reflect *either* disease trajectory *or* REM-suppression from the prior doxepin/trazodone stack. Switching to a REM-enhancer introduces the possibility of dream-enactment episodes returning. No direct clinical evidence either way. **This is the single most uncertain part of the comparison.**
- **SELENADE depression-improvement signal doesn't apply cleanly.** The 2025 open-label pilot showed HAM-D-17 improvement, but in patients with baseline scores around 13.6 — far below Dad's PHQ-9 of 24. No control group. Should not materially weight the choice.
- **Cost.** ~\$300/month not covered by insurance. Family has dismissed this as a barrier; flagging for completeness.

- **Laurin 2022 found anticholinergic burden NOT associated with post-ECT cognitive impairment** in 42 TRD patients. This counterweights the categorical Lancet 2025 avoidance recommendation for the specific pre-ECT and ECT-course window.
- **Doxepin 6 mg has FDA approval specifically for insomnia.** At this dose it's primarily a selective H1 antihistamine; the anticholinergic, serotonergic, and noradrenergic effects that drive antidepressant-dose doxepin's side-effect profile are minimal at 6 mg.
- **REM suppression may help maintain current RBD quiescence.** Both doxepin (at antidepressant doses; minimal at 6 mg) and trazodone are mild REM suppressors. If the recent decline in Dad's RBD episodes is partly pharmacologic rather than purely disease trajectory, this regimen continues that suppression. Theoretical, but the converse case for lemborexant is also theoretical.
- **No new variable pre-ECT.** ECT centers prefer stable regimens entering induction; this preserves that.
- **Lower placebo risk.** Dad knows this regimen; the anxiety around "will the new thing work" doesn't apply.
- **Zero cost barrier.** Both generic; trivial cost.

CONS

- **"Okay not great" is the realistic ceiling.** 5.5–6 hours, no dream recall, wakes unrested, can't nap during the day. The known floor isn't a robust one; this regimen has not produced the sleep Dad actually needs. The literature predicts roughly this outcome — sedating antidepressants improve sleep continuity but residual sedation can impair subjective refreshment.
- **Lancet Neurology 2025 categorically recommends avoiding anticholinergic TCAs in Lewy-spectrum disease.** No dose threshold specified. Even though doxepin 6 mg is quantitatively far below documented-harm exposure, the guideline-level recommendation is directional regardless of dose math.
- **Additive sedation, orthostatic hypotension, and fall-risk concerns over 2–6 weeks.** Two sedating sleep agents + Cymbalta in a 76-year-old with sinus bradycardia (HR 49–53), low-normal BP (112/58), and documented gait abnormality is a clinically meaningful fall-risk profile over the medium-term window — not just one night.
- **Trazodone monotherapy evidence is weak.** AGS Beers Criteria 2023 companion: "insufficient evidence to recommend trazodone" in older adults. The combination's value rests on doxepin's evidence + clinical practice patterns for trazodone, not RCT support for the combo specifically.

- **Not anxiolytic per se.** Lemborexant blocks the wake signal but does not directly reduce anxiety. If Dad goes to bed anxious, the orexin blockade alone may not be enough to override it.
- **Starting a new agent 2–4 weeks pre-ECT is itself a variable.** ECT centers typically prefer stable regimens entering induction. If something happens during the ECT course, attribution becomes harder.

- **Dad attributed "agitation" to doxepin.** The published literature has no documented paradoxical-agitation pattern at 6 mg, so this is most likely attribution error or nocebo — but Dad's subjective experience is itself a clinical fact that affects adherence and may worsen the anxiety state directly.
- **Doesn't advance the long-term regimen.** U-M will eventually want a sleep stack going into long-term care; whether doxepin stays depends on the Movement Disorders workup. Choosing this now defers that transition.
- **Three concurrent sleep agents (counting melatonin) is more attribution-noisy** than one. If anything changes in Dad's state — better or worse — it's harder to know which agent is responsible.

How they compare at a glance

Dimension	Option A — Lemborexant + melatonin	Option B — Doxepin + trazodone + melatonin
Personal data for Dad	None yet — untried	5.5–6 hrs sleep, no dream recall, wakes unrested, no serious AEs
Trial evidence in elderly	Robust — SUNRISE-1, SUNRISE-2 ≥ 65 subgroup (Arnold 2024); SELENADE 2025 depression-comorbidity signal is small and uncontrolled, applies poorly at Dad's severity	Doxepin 6 mg robust; trazodone 50 mg weak (AGS Beers 2023 companion: "insufficient evidence"); combination not formally studied
Time to plateau	1–2 weeks — rapid plateau, not progressive. 1–2 week trial sufficient to assess	Already at steady-state in Dad's prior experience
Sleep maintenance (2 AM problem)	17-hr half-life — drug still active overnight; WASO reduction is the established advantage	Doxepin's primary benefit is sleep maintenance; combination covers both halves; Dad still wakes unrested on it
Anticholinergic burden over 2–6 weeks	None	Quantitatively $\sim 2\%$ of demonstrated-harm threshold (Gray 2015); but Lancet 2025 categorical "avoid TCAs in Lewy-spectrum" guideline at any dose
RBD / REM effect	Increases REM; could unmask currently-quiescent RBD episodes. No direct clinical data.	Both agents REM-suppressing; if RBD quiescence is partly pharmacologic, this regimen maintains it. No direct clinical data.
Drug interactions with Cymbalta	Clean — no CYP2D6, no additive serotonergic, no orthostasis	Modest CYP2D6 elevation of doxepin (clinically negligible at 6 mg); additive orthostasis and sedation
Fall risk in Dad's profile	Low	Clinically meaningful over 2–6 weeks given sinus bradycardia (HR 49–53), low-normal BP (112/58), gait abnormality
Dependence risk	None — FAERS lowest of any insomnia agent	None — both very low abuse potential
Anxiolytic effect	None directly — blocks wake signal but not anxiety	Modest — doxepin's antihistamine calms; trazodone's 5-HT _{2A} /alpha-1 antagonism is mildly anxiolytic
Cost (Dad-side)	$\sim \$300/\text{month}$ not covered	Both generic; trivial cost
Number of agents	1 (+ melatonin)	2 (+ melatonin)
Pre-ECT logic	Adds a new variable 2–4 weeks before induction; ECT centers prefer stable regimens. Laurin 2022: anticholinergic burden does NOT predict post-ECT cognitive impairment, weakening one reason to switch pre-ECT.	No new variable; clean stability entering induction. Laurin 2022 supports the safety case.
First-night risk	Moderate — untried; nocebo risk if framed poorly	Low — known-tolerated; predictable

Clear stacking rules — what NOT to combine

DO NOT — THESE COMBINATIONS ARE NOT THE PLAN AND SHOULD NOT BE DRIFTED INTO

- **DO NOT take lemborexant AND trazodone the same night.** They are alternatives, not stack-mates. If choosing Option A, both doxepin and trazodone are off the regimen. If choosing Option B, lemborexant is off. Stacking two sedating agents with different mechanisms adds risk (orthostatic hypotension, falls, additive sedation) without an evidence base for the combo.
- **DO NOT take lemborexant AND doxepin the same night.** Same logic — Option A is lemborexant only (+ melatonin). Adding doxepin reintroduces anticholinergic load and undermines the rationale for switching.
- **DO NOT take only one of doxepin or trazodone (Option B is the pair).** Trazodone monotherapy has weak evidence in elderly (AGS Beers: "insufficient evidence"). Doxepin monotherapy at 6 mg works on sleep maintenance but is light on sleep onset. The Option B value is the combination; don't degrade it by dropping one component.
- **DO NOT fold Depakote into the sleep regimen question.** Depakote is a separate decision being discussed for acute anxiety. Whether it goes on board, at what dose, and for how long is a question for the U-M conversation. Treating Depakote as part of the sleep stack confuses two independent questions.
- **DO NOT take Ativan as a scheduled nightly dose.** One emergency use is fine; nightly use establishes a dependence pattern that becomes hard to undo. If Ativan ends up being needed multiple nights in a row, that's a signal to escalate to U-M faster, not to add it to the routine.
- **DO NOT stack a second sleep aid mid-night.** If the chosen regimen doesn't work and Dad wakes at 12 or 1 AM, do not pile on a different sleep aid. Stimulus control + acceptance is the right response, not pharmacologic escalation in the middle of the night.
- **DO NOT skip the Cymbalta morning dose to "reduce serotonergic load."** The taper is paused. Dad is at 60 mg by design. Skipping a dose restarts the destabilization the pause is meant to prevent.

DO — THESE COMBINATIONS ARE THE PLAN AND ARE EVIDENCE-SUPPORTED

- **DO take Cymbalta 60 mg AM** per the paused-taper schedule. Sleep regimen is separate.
- **DO take melatonin 10 mg HS** with either Option A or Option B. Negligible interactions; modest sleep-onset support.
- **DO commit to one regimen for a fair trial window.** Option A needs 1–2 weeks to evaluate (lemborexant's effect plateaus within the first week, so 1–2 weeks is sufficient); Option B already has Dad's prior data and can be confirmed in 1–2 nights. Switching mid-trial defeats the assessment.
- **DO use Ativan PRN as a single-dose rescue if truly needed** — once. Note the dose, time, and effect. Don't escalate to scheduled use without escalating to U-M.
- **DO track sleep nightly in writing** — onset, awakenings, total time, subjective quality, AM mood. The data is what informs the next decision.
- **DO treat Depakote, the Cymbalta taper, and the sleep regimen as three independent questions** — each gets discussed with U-M on its own merits.

Tonight specifically

Tonight is the first night of whichever option is chosen. If Option A, it's the first lemborexant trial. If Option B, Dad goes back to a known stack. Either way, the framing for Dad pre-bed matters more than usual, given the acute anxiety state today.

FOR MOM TO DAD — IF OPTION A IS CHOSEN

This is the medication Julane recommended. It works differently from doxepin and trazodone — instead of making you sleepy, it turns down the wake signal. It lasts the whole night, so if you wake up at 2 AM, the medicine is still working — give it a few minutes. We're trying it for the first time tonight, so it may take a few nights to know how well it works for you. One night doesn't tell us much; we'll know more after three or four. If you really can't sleep, we have Ativan as a one-time backup.

FOR MOM TO DAD — IF OPTION B IS CHOSEN

We're going back to the regimen you were on before — doxepin, trazodone, melatonin. You know how this feels and we know it gives you a decent night's sleep. We're not committing to it long-term — when U-M gets involved we'll revisit. Tonight is about familiarity and not adding a new variable on a hard day.

Trial window and explicit switch criteria

- **Whichever option is chosen, give it a fair trial window.** Lemborexant plateaus quickly — 1–2 weeks is sufficient to assess (per published efficacy trajectory). Dad already has data on Option B so 1–2 nights confirms whether it's consistent with his prior experience.
- **What "working" looks like:** ≥ 6 hours of total sleep, falls asleep within ~ 30 min, no more than 1–2 awakenings of ≤ 20 min each, wakes feeling at least somewhat rested, no new daytime symptoms (worse anxiety, motor changes, cognitive cloudiness beyond baseline).
- **What "not working" looks like:** ≤ 5 hours total, multiple long awakenings, AM anxiety spike, new daytime symptoms.
- **Switch from Option A → Option B if:** (a) emergence of nightmares, vivid dreams, or RBD-episode return — stop lemborexant; (b) no improvement in subjective sleep quality over Dad's prior baseline after 1–2 weeks (10 mg dose escalation is also on the table before reverting); (c) U-M ECT team requests it.
- **Switch from Option B → Option A if:** (a) worsening orthostatic symptoms or falls; (b) increased morning sedation interfering with daytime function; (c) the U-M Movement Disorders evaluation upgrades the prodromal-Lewy-spectrum picture and U-M recommends getting off doxepin; (d) the ECT team requests it.
- **If anything new appears** — dream enactment / RBD escalation, new motor symptoms, paradoxical agitation, GI changes — note it. These observations feed the Movement Disorders evaluation directly.
- **If Ativan is used:** note the dose, time, and how much it helped. One night is fine; multiple nights in a row is a signal to escalate to U-M, not to extend the pattern.
- **Bring the data to Maxner.** Whichever regimen is on board going into the Maxner callback becomes a known quantity for the U-M plan. Tracked nightly data makes the conversation more informative.

Bottom line

The published evidence does not clearly favor either regimen. Both are reasonable; the magnitude of the advantage for either side is small. Option A's modest evidence-supported edges are in prodromal-Lewy-spectrum safety direction (Lancet 2025

guideline-level), drug-interaction cleanliness with Cymbalta, and lower fall-risk profile in Dad's bradycardic + low-BP + gait-abnormality setting. Option B's modest practical edges are in known tolerability, RBD-quiescence maintenance via REM suppression (theoretical), no new variable pre-ECT (with Laurin 2022 supporting the safety case), and zero cost barrier.

Dad and Mom are the right people to make this call. The job of this handout is to ensure the call is deliberate, that whichever option is chosen gets a fair 1–2 week trial with the explicit switch criteria above, and that the family doesn't drift into a regimen no one chose — by stacking lemborexant on top of doxepin + trazodone, by adding Depakote into the sleep question, or by escalating Ativan into nightly use without anyone noticing.

The medium-term frame matters because ECT is 2–4 weeks out. Whichever stack is on board going into Maxner's intake conversation becomes the starting point for U-M's plan. That's worth choosing on purpose rather than by accumulation.